MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 23 OF 28

MFT 722

Professional Development and Self-Care

14 teaching weeks · 1 practicals · 0 worked cases · 1 figures

ELECTIVES

1.71 units · Prerequisite: MFT 706

> “This is not a wellbeing add-on at the end of training.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 722 — Professional Development and Self-Care

MSc Marriage and Family Therapy · Elective · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 706

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	**Professional Development and Self-Care**	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. This is not a wellbeing add-on at the end of training. It is the course that decides whether you are still practising in fifteen years, and whether the practising is any good. It treats the therapist's own life as the primary instrument, and professional development as a discipline rather than an ambition.

How to use this book

Every week has a **professional** task and a **personal** one. Do both. The course is assessed largely on a portfolio, and a portfolio written retrospectively in the final week will be visible and weak. Keep a weekly entry; it takes eight minutes.

1. Course description

The course covers: the person of the therapist; burnout, compassion fatigue and vicarious trauma; boundaries and availability; personal therapy; reflective practice and supervision; continuing professional development; career pathways and private practice fundamentals; money, contracts and pricing; professional identity and belonging; and sustainable practice in resource-limited settings.

2. Learning outcomes

#	Outcome	Assessed
LO1	Assess your own burnout and vicarious trauma risk using structured tools	Portfolio
LO2	Design a personal and professional self-care plan that survives a busy month	Plan
LO3	Set and maintain boundaries with clients, colleagues and institutions	Viva
LO4	Plan continuing professional development against a competency framework	CPD plan
LO5	Explain the fundamentals of ethical private practice (contracts, fees, records)	Exam
LO6	Use reflective practice and personal therapy appropriately	Portfolio
LO7	Articulate a five-year professional trajectory	Presentation

3. Assessment

Component	Weight	Due
Reflective portfolio (10 entries)	35%	Fortnightly
Personal and professional sustainability plan (1,500 words)	25%	Week 10
Practice readiness viva (20 min)	25%	Week 13
CPD and career presentation (10 min)	15%	Week 14

4. The fourteen-week plan

Week	Topic	Task
1	The person of the therapist	Therapy autobiography (private)
2	Burnout: what it is and how it develops	Maslach self-assessment
3	Compassion fatigue and vicarious trauma	Symptom inventory
4	Boundaries: time, contact, disclosure, gifts	Boundary audit
5	Personal therapy and its place in training	Decide and plan
6	Reflective practice: models and habits	Weekly reflection structure
7	Supervision as a career-long practice	Supervision plan
8	CPD and competency frameworks	CPD gap analysis
9	Career pathways in MFT	Pathway map
10	**Sustainability plan due**; money, fees and contracts	Fee and contract design
11	Private practice fundamentals	Practice blueprint
12	Professional identity, community and belonging	Professional network plan
13	**Practice readiness viva**	Assessed
14	**CPD and career presentation**	Assessed

Week 1 — The person of the therapist

The premise. You cannot separate the clinician from the person. Your history shapes what you notice, what you avoid, whom you like, and where you lose your balance.

The "therapy autobiography". Write, privately, about: your family of origin; the rules you absorbed; the losses; the roles you played; what you learned about conflict, intimacy, money and authority; and why you chose this work. Do not submit it. Do keep it, and revisit it in three years.

Common motivations and their risks.

Motivation	Strength	Risk
"I want to help people"	Compassion	Rescuing; over-functioning
"I want to understand my family"	Insight	Using clients to solve your own history
"People tell me I'm easy to talk to"	Alliance skill	Difficulty setting boundaries
"I survived something"	Credibility	Over-identifying; avoidance of certain material
"I want status/expertise"	Drive	Distance; difficulty with not-knowing

The professional requirement. Know your history well enough that you can tell the difference between your client's material and your own reaction. Personal therapy and supervision exist for this, not because something is wrong with you.

Week 2 — Burnout

Definition (Maslach): a syndrome of **emotional exhaustion**, **depersonalisation/cynicism**, and **reduced sense of accomplishment**, arising from chronic occupational stress.

The six drivers (Maslach & Leiter) — all of them organisational:

1. Workload
2. Control (or lack of it)
3. Reward (financial, social, intrinsic)
4. Community
5. Fairness
6. Values (mismatch between what you believe and what the system asks)

The clinically important point. Burnout is not a personal failing and cannot be solved by yoga. If the caseload is impossible, the supervision absent and the values compromised, self-care is a distraction from the real problem.

Self-assessment. Use the Maslach Burnout Inventory or the freely available Copenhagen Burnout Inventory. Score yourself honestly; write one paragraph on which of the six drivers is most active in your current situation.

Week 3 — Compassion fatigue and vicarious trauma

Three distinct concepts.

Concept	Definition	Signature
Burnout	Exhaustion from workload	"I am too tired to care"
Compassion fatigue	Reduced empathy and satisfaction from caring	"I used to feel for people; now I don't"
Vicarious trauma	Changes to beliefs and inner world from exposure to others' trauma	"The world is not safe; people are dangerous"

Signs to monitor in yourself. Intrusion of client material; nightmares; avoidance of certain cases; cynicism; irritability at home; numbness; loss of hope; over-involvement; boundary erosion; sleep and appetite change; increased alcohol use.

Protective factors with evidence. Supervision that addresses the emotional impact; caseload diversity; control over scheduling; peer support; meaning-making; sleep and exercise; personal therapy; and organisational support.

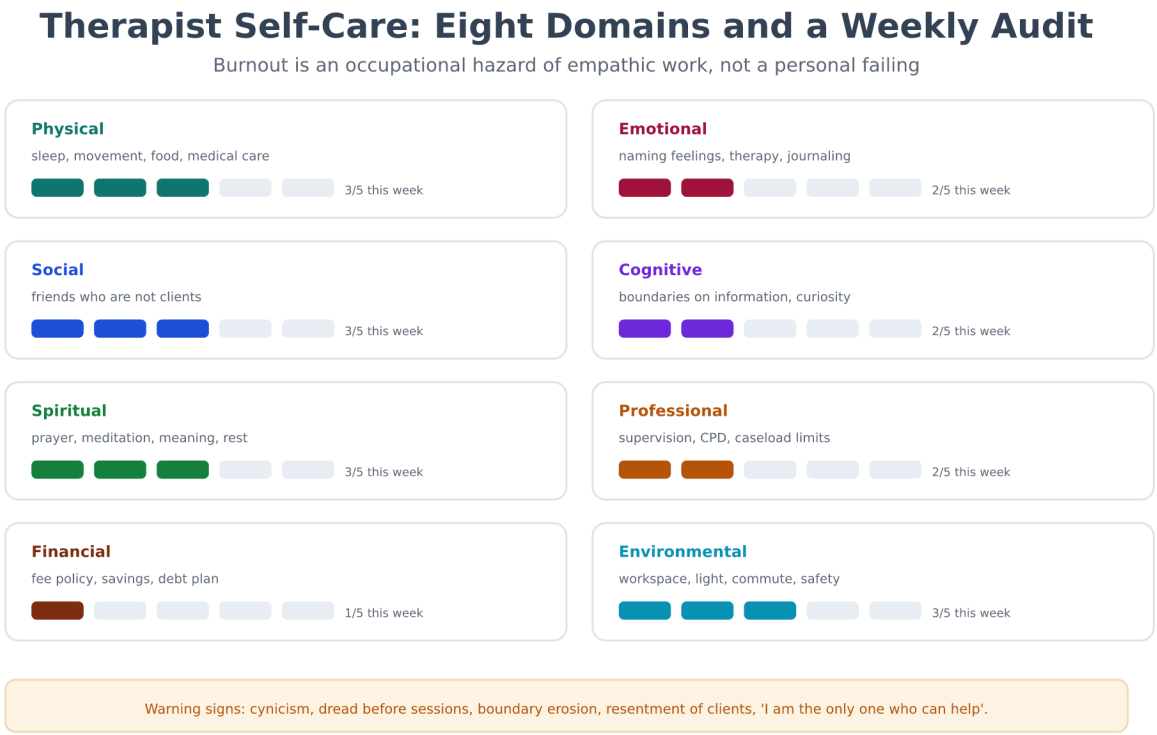


Figure 3.1 — Self-care audit across eight domains

TASK

Complete the eight-domain audit. Rate each domain 0–5 for last week. Identify the lowest two and write one change for each that is small enough to happen on a Tuesday.

Week 4 — Boundaries

The boundaries that matter most in early practice.

Boundary	The trap	A workable standard
Time	Sessions over-run; you arrive early, leave late	Fixed session length; 10 minutes between clients
Contact	Clients text at night; you reply	Written policy; emergency route; no personal number
Disclosure	You share to reassure	Disclose for the client's benefit, rarely, and briefly
Gifts	Refusing feels rude	Accept small cultural gifts; discuss significant ones
Social media	Client requests; you see their posts	Clear policy; never search for clients
Dual roles	Church, community, family ties	Disclose, discuss, refer if unmanageable
Emotional labour	You become the family's friend	Warmth plus frame; endings are explicit

The test. Not "was it kind?" but "whose need did it serve, and would I be comfortable explaining it in supervision?"

Week 5 — Personal therapy

Requirements vary. Many training programmes require a minimum number of personal therapy hours. Treat the requirement as a floor, not a target.

What it is for. Not to become "fixed", but to know your patterns well enough to notice them in the room; to have a place to process work that is not your supervisor's job; and to experience being a client.

What to look for in a therapist. A different orientation from yours (to see another way of working); a recommendation from someone you trust; a clear contract; and someone you can be honest with.

The cost problem. Therapy is expensive, and trainees are often poorly paid. Options: reduced-fee schemes, group therapy, university counselling services, peer consultation groups, and budgeting it as a professional expense rather than a luxury.

Week 6 — Reflective practice

Reflection is a method, not a mood. Use a structure.

The Gibbs cycle, applied to a session: description → feelings → evaluation → analysis → conclusion → action plan.

Six questions to answer weekly.

1. What happened that surprised me?
2. What did I feel, and where in my body?
3. What did I avoid?
4. What did I do that worked?
5. What would I do differently?
6. What do I need to bring to supervision?

Writing. Keep one page per week. Do not write case details in a personal journal that is not secure; use initials or pseudonyms, and never store client-identifying material outside the clinical record.

Week 7 — Supervision as a career-long practice

The norms. One hour of supervision for every five hours of clinical work is the common standard; more for complex cases and for trainees.

How to use supervision well.

- Arrive with an agenda and one clear question.
- Bring the case you least want to bring.
- Say what you felt, not only what you did.
- Ask for observation, not only opinion ("what would you have said?").
- Record what you will do differently, and report back next time.

Peer consultation groups. A complement, not a substitute: 3–5 clinicians, monthly, structured, with a confidentiality agreement and a rotating facilitator.

Week 8 — Continuing professional development

Frameworks. Most professional bodies require annual CPD across domains: clinical practice, supervision, ethics, research, teaching, management. Map your year against the domains rather than collecting certificates.

A CPD plan that works.

1. Identify three gaps from your competency self-assessment.
2. For each, specify an activity, a date, and an evidence of completion.
3. Include one activity that stretches you (a new modality, a research skill, a leadership role).
4. Review every six months; adjust.

Beware CPD theatre. Ten webinars on topics you already know count for nothing. One supervised course in an area of genuine weakness counts for a great deal.

Week 9 — Career pathways

Pathway	What it needs	What it gives	What it costs
Clinical specialist	Depth in a modality; supervision; certification	Expertise, referrals	Narrowing
Public sector clinician	Systems navigation; resilience	Security, breadth	Bureaucracy, heavy caseloads
Private practice	Business skills, capital, network	Autonomy, income potential	Isolation, income volatility
Academic/teaching	Research, publication, teaching	Influence, stability	Bureaucracy, distance from clinic
Supervision/consultation	Experience, accreditation	Leverage, variety	Responsibility for others' practice
NGO/humanitarian	Flexibility, languages, resilience	Meaning, breadth	Insecurity, exposure to trauma
Policy/advocacy	Writing, networks, patience	Scale of impact	Slowness, compromise

The honest question. Which of these fits the life you actually want — including your family, your health, and your finances — not only the career you imagined at 22?

Week 10 — Money, fees and contracts

Fees are clinical decisions. Set them deliberately; publish them; apply them consistently; review annually.

A fee policy covers: the fee; session length; payment method and timing; late cancellation; non-attendance; sliding scale criteria (decided in advance); annual review; and what happens to outstanding balances.

Contracts. Even a one-page agreement prevents most disputes: parties, purpose, confidentiality and limits, fees, cancellation, contact policy, complaints route, and signatures.

The uncomfortable truths.

- Charging is not exploitation; under-charging until you resent clients is.
- A sliding scale with no criteria becomes arbitrary and unfair.
- Chasing unpaid fees is a clinical conversation, not an administrative one.
- You will need to budget for tax, insurance, supervision, CPD and sick leave.

Week 11 — Private practice fundamentals

The blueprint. Location and room (privacy, sound, waiting area, accessibility); legal structure and registration; professional indemnity insurance; record-keeping and data protection; booking and payment systems; referral network; risk and emergency protocols; supervision arrangement; and a marketing approach that is ethical and honest.

Starting safely. Keep a part-time employed post while you build; take a business course or find a mentor; decide your niche only after 50 cases, not before; and never practise without supervision, however experienced you are.

Isolation is the occupational hazard. Build a peer group before you need one.

Week 12 — Professional identity and belonging

Belonging matters. Clinicians who are connected to a professional community report less burnout, better decision-making, and greater persistence.

How to build it. Join a professional association; attend one conference a year; contribute to a peer group; teach one workshop; write one short piece; mentor one junior colleague.

Identity in a contested field. Family therapy sits between psychology, psychiatry, social work and counselling. Be clear about who you are and what you offer, without disparaging

others: "I am a systemic therapist; my expertise is in relationships. I work alongside your psychiatrist, not instead of them."

Week 13 — Practice readiness viva

Twenty minutes. Questions drawn from: boundary scenarios, burnout scenarios, ethical dilemmas, fee conversations, supervision difficulties, and career planning. Assessed on judgement, honesty, and the ability to name your own limits.

Week 14 — CPD and career presentation

Ten minutes on where you are, where you are going, what you need, and what you will stop doing. Include one commitment to your own sustainability that is specific, dated, and verifiable.

Glossary

Burnout — exhaustion, cynicism and reduced efficacy from chronic occupational stress. **Compassion fatigue** — reduced empathy and satisfaction resulting from prolonged caring. **Compassion satisfaction** — the positive counterpart: meaning and pleasure derived from helping. **CPD** — continuing professional development. **Reflective practice** — structured examination of one's own work and reactions. **Vicarious trauma** — transformation of the therapist's inner world through exposure to others' trauma.

Self-assessment

Multiple choice

1. Burnout is primarily caused by: (a) personal weakness (b) chronic occupational stressors (c) too much supervision (d) client resistance

2. Vicarious trauma is characterised by: (a) tiredness (b) changes to beliefs and inner world (c) low fees (d) poor timekeeping

3. A workable contact boundary is: (a) giving your personal number (b) a written policy with an emergency route (c) replying at night (d) no policy

4. Personal therapy in training is best understood as: (a) a punishment (b) a professional requirement and an investment (c) unnecessary (d) a substitute for supervision

5. The recommended supervision ratio is roughly: (a) 1 hour per 20 client hours (b) 1 hour per 5 client hours (c) 1 hour per year (d) no standard

6. A useful CPD plan starts from: (a) available webinars (b) a competency gap analysis (c) cheap courses (d) colleagues' choices

7. Fee policies should be: (a) improvised (b) written, published and applied consistently (c) secret (d) identical everywhere

8. Peer consultation groups: (a) replace supervision (b) complement supervision (c) are unnecessary (d) are illegal

9. Reflection is best done: (a) once a year (b) weekly with a structure (c) only in crisis (d) verbally only

10. Private practice should ideally begin: (a) with no supervision (b) alongside employed work with supervision in place (c) with maximum debt (d) without insurance

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Name the three dimensions of burnout and the six organisational drivers.
2. Distinguish burnout, compassion fatigue and vicarious trauma.
3. List five boundaries and the standard you will use for each.
4. What are the six questions of a weekly reflection?
5. Outline the components of a fee policy.

Essay (choose one, 1,500 words)

1. "Self-care is a professional obligation, not a luxury." Discuss with reference to the evidence on burnout.
2. Design a sustainable practice model for a newly qualified family therapist in a low-resource setting.
3. Analyse your own family-of-origin patterns and their likely effect on your clinical work.

Further reading

- Norcross, J. C., & Guy, J. D. (2019). Leaving It at the Office (2nd ed.). Guilford.
- Figley, C. R. (Ed.). (1995). Compassion Fatigue. Brunner/Mazel.
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- Skovholt, T. M., & Trotter-Mathison, M. (2014). The Resilient Practitioner (3rd ed.). Routledge.
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Next: MFT 723 — Special Populations in Therapy